Anticipatory Guidance in Adolescents

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Definition

- ☑ Anticipatory guidance is defined as proactive counseling that addresses the significant physical, emotional, psychological, and developmental changes that will occur in adolescents during the interval between health supervision visits.
- ☑ Anticipatory guidance prepares the adolescent and their families for what to expect next in their child's development. It addresses protective factors aimed at preventing physical and mental health problems.

Burden of the Problem

India has the largest young population in the world comprising of 253 million adolescents in the age group 10–19 years. In fact, every fifth person in India is an adolescent.

As rapid changes occur in body and mind (psychosocial development), adolescents have an identity crisis and tend to feel overly emotional. Mood swings commonly occur among teenage boys and girls.

Children who hit puberty feel uncomfortable with their bodily changes which results in self-consciousness and anxiety. Feelings of inferiority or superiority may arise at this time.

As the hormones surge, sexual feelings arise! Thoughts about sex can trigger a sense of guilt in an adolescent.

Parents of adolescents also find the transition from childhood to adolescence as difficult times. They experience increased parent–adolescent conflict, stress, and emotional distancing with their children. Anticipatory guidance can successfully improve parenting practices, family conflicts, and adolescent behavioral and mental health.

Research

The writing committee contacted 30 adolescents and their guardians telephonically to flag the prominent areas requiring anticipatory guidance. The purpose was to: (1) Explore the problem areas associated with parenting an adolescent and (2) identify burning topics for guidance to adolescents and their families.

- ☑ Parents wanted guidance on communication (65%) and guidance on conflict management (63%). Peer affiliation (55%), mobile overuse (60%), and academic performance (91%) were major concerns.
- ☑ 45% adolescents sought guidance on mental health where academic performance and parental expectation was a major issue.
- ☑ 74% adolescents had questions regarding alcohol and drugs.
- ☑ Almost all adolescents expressed eagerness for guidance on sexuality (87%).

Our study finds that adolescents expect to find help in their pediatrician's office and therefore must not feel abandoned when their need for comprehensive services is the greatest.

Parents too identify difficult areas associated with raising adolescents and are open to receiving guidance from a pediatrician or through written literature.

Issues which need anticipatory guidelines			
Preventive health issues: ☑ Safety issues ☑ Substance abuse ☑ *Sexuality:* Relationships/ abuse, etc. ☑ Suicidal ideation	*Healthy lifestyle:* ☑ Body image issues ☑ Diet ☑ Sleep ☑ Exercise ☑ Screen time ☑ Hygiene ☑ Immunization	Social media and Internet safety	*Emotional behavior:* ☑ Normal emotional changes ☑ Parenting ☑ Anxiety ☑ Depression

HEEADSSS assessment[1] is extremely helpful in pointing out areas where anticipatory guidance may be required. The attending pediatrician can give guidance to both caretakers and adolescents during the annual health checkup in a school or while attending to a sports injury. They are also available for anticipatory guidance when they come for the booster immunizations of human papillomavirus (HPV) or tetanus, diphtheria, and pertussis (Tdap) at 10 years.

- ☑ **Body image issues:** Not all of us are completely satisfied with the way we look and feel about ourselves. For some adolescents, this dissatisfaction may become a persistent worry. Such children are obsessed with counting calories, taking up crash diets, or talking negatively about their body.

  The pediatrician can give guidance about normal physical growth and development (including changes at puberty) to the adolescents in well clinics. Anticipatory guidance to parents also includes information about normal physical growth and development of their child. Absence of breast development by 12 years and absence of menses by 16 years are indications to seek medical advice. In boys, if there is no testicular growth by 13 years, they should seek medical help.

  The adolescent should be advised to focus on what his body can do, rather than how his body looks. He should spend time on interests and activities that make him feel good. The whole family should be counseled that praising their child for who he is and what he can do matters more than his size or shape. They should be proud of him for things that are not related to appearance.
- ☑ **Parenting:** Each family is unique and so is each adolescent! Teens raised by positive parenting do better at school, deal better with conflicts, have lower rates of depression, are less likely to engage in risky behavior, and have better social skills.

  Parents should be explained about authoritative parenting where the parent–child relationship works on unconditional love, negotiation skills, and effective communication. A simple sentence, "I see you seem to be upset about something. Can we talk about it?" can ease the adolescent's distress. Imparting life skills and spirituality are protective factors in parenting.
- ☑ **Safety:** In 2019, nearly 100,000 adolescents (10–19 years) died as a result of road traffic accidents (NCRB Report). Pediatrician should discuss risk of speeding, accidents, and safety measures, e.g., abiding by traffic rules, use of seat belt, helmets, etc.
- ☑ **Substance abuse:** Provider can introduce the topic during psychosocial interviewing by asking about substance use in peers and skillfully steer the conversation to harmful effects of tobacco, smoking, alcohol, drugs, etc. An adolescent with substance abuse may need psychiatric and pharmacological therapy.
- ☑ **Sexuality:** Talk the talk before they walk the walk! Provider should talk about menses, night falls, love, crush, loyal relationships, abstinence, sexually transmitted diseases (STDs), and if adolescent is sexually active, talk about contraceptives.
- ☑ **Suicidal ideation:** Provider should teach adolescents to recognize the red flag signs of suicidal ideation (anhedonia, giving away prized possessions, sudden academic deterioration, or a direct expression of death wish) in their peers and alert the school authorities immediately. Such patients should immediately be referred to the psychiatrist.

- ☑ **Mental health:** Mental health conditions account for 16% of the global burden of disease and injury in people aged 10–19 years. Half of all mental health conditions such as ADHD, anxiety, depression, and schizophrenia begin before the age of 14 years and three quarters of mental health disorders begin before the age of 24 years. However, most cases are undetected and untreated. Adolescents and their parents must be made aware regarding importance of early treatment and its lifetime implications.
- ☑ **Emotional disorders:** Learning issues at school or relationship issues such as divorce of parents, sibling rivalry, or death in the family may cause additional stress in adolescents. Experimenting with drugs/alcohol may be a cause of concern as well. Anxiety disorders, bipolar disorders, conduct disorders, eating disorders, obsessive-compulsive disorder (OCD), and psychotic disorders are quite common in adolescents.

  Parents should be given anticipatory guidance regarding signs and symptoms and depending on the diagnosis, the adolescent may be treated with: Cognitive behavior therapy (CBT), family therapy, or group therapy.
- ☑ **Balanced diet:** Iron-deficiency anemia was the second leading cause of years lost by adolescents to death and disability in 2019. Incidence of obesity has doubled in last 5 years. Providers should discuss about importance of healthy and balanced diet and counsel to avoid JUNCS[2] and limit to one serving/week.
- ☑ **Sleep:** Adolescents need a minimum of 8 hours sleep to help them perform optimally. An adolescent with delayed sleep wake pattern will fall asleep well after midnight and will have difficulty getting up in time for school. Adolescents should be advised to develop a consistent sleep schedule and good sleep habits which include going to bed and waking up at the same times (including weekends), avoiding caffeinated products or other stimulants before bedtime, maintaining a cool, quiet, and comfortable bedroom, and avoiding screen viewing before bedtime.
- ☑ **Exercise:** Anticipatory guidance regarding exercise includes moderate exercise at least three times a week for 30 minutes to 1 hour at a time. Explain that being sedentary carries a strong risk for chronic diseases such as obesity, high blood pressure, diabetes, and heart disease later in life.
- ☑ **Screen time:** The screen time of adolescent should not be >2 hours a day. Rule of 20 × 20 × 20 (20 minutes of screens followed by 20 seconds of looking at objects 20 feet away) should be explained.[3]
- ☑ **Hygiene issues:** Personal hygiene is particularly important during puberty. Basic hygiene involves washing hands, bathing, brushing teeth, and covering the mouth while coughing. Tell the adolescent that your body is exposed to many outside germs and viruses that can make you sick. Simply washing hands with soap can significantly reduce risk of transmission of contagious infections as witnessed in COVID-19. Brushing teeth twice a day prevents dental caries and bad breath. Talk to the girls about menstrual cycle and menstrual hygiene.

  Good personal hygiene also has a positive impact on self-esteem, confidence, and body image.

Anticipatory Guidance

- ☑ **Social media and Internet:** Provider should talk to adolescent about media literacy—"watch carefully, think critically." Questions to be answered before posting on the Internet: (1) Is it true?, (2) Is it hurtful?, (3) Is it illegal?, and (4) Is it necessary?

Indian Academy of Pediatrics has come up with community awareness programs such as "Mission Kishore Uday," "Awesome AYA," and "Awesome AYA for schools" where all stakeholders, viz., adolescents, parents, teachers, and pediatricians have been imparting knowledge about the issues involving anticipatory guidance. The Academy has also recently published the 105 parenting guidelines[4] which serve as ready reckoners for anticipatory guidance.

The writing committee suggests that anticipatory guidance for adolescents and their parents must be integrated into universal practice in healthcare settings.

Further Reading

- ☑ Gupta P, Shah D, Bedi N, Galagali P, Dalwai S, Agrawal S, et al. Indian Academy of Pediatrics Guidelines on Screen Time and Digital Wellness in Infants, Children and Adolescents. Indian Pediatr. 2022;59(3): 235-44.
- ☑ Gupta P, Shah D, Kumar P, Bedi N, Mittal HG, Mishra K, et al. Indian Academy of Pediatrics Guidelines on the Fast and Junk Foods, Sugar Sweetened Beverages, Fruit Juices, and Energy Drinks. Indian Pediatr. 2019;56(9):849-86.
- ☑ IAP 101 Parenting guidelines.
- ☑ Klein DA, Goldenring JM, Adelman WP. HEEADSSS 3.0; The psychosocial interview for adolescents updated for a new century fueled by media. Contemporary Pediatrics. 2014. pp. 16-28.

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Autism Spectrum Disorders

Lead Author

Samir Hasan Dalwai

Co-Authors

Shabina Ahmed, CP Ravikumar

Under the Auspices of the IAP Action Plan 2022

Remesh Kumar R
IAP President 2022

Upendra Kinjawadekar
IAP President-Elect 2022

Piyush Gupta
IAP President 2021

Vineet Saxena
IAP HSG 2022–2023

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IAP Standard Treatment Guidelines Committee

Chairperson

Remesh Kumar R

IAP Coordinator

Vineet Saxena

National Coordinators

SS Kamath, Vinod H Ratageri

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Krishna Mohan R, Vishnu Mohan PT

Members

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